NurturePath AI

USER GUIDE + COMPLETION ROADMAP

Care that follows her home.

What has been built. What you can do.
What remains. How to finish it.

Working local MVP

NurturePath AI helps workers and clinics coordinate maternal follow-up: enrolment with consent, appointments, contact outcomes and referrals that are tracked through closure.

> **Current operating boundary**
> This release is for fictional adult records and simulated reminders. Real-patient onboarding is blocked. Clinical, legal, partner, security, operations and pilot approvals remain pending.

Prepared 10 October 2026 • Based on the three supplied project PDFs, implemented screens and recorded verification. Includes the browser-extension hydration fix.

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01 / START HERE

Open your working demo

On this computer, open http://localhost:3200. The app was started locally; if it has stopped, open PowerShell in the project folder and run:

```
docker compose up -d db
npm run dev
```

Leave that terminal running. Press `Ctrl+C` there to stop the app. The database can remain running. Ports 3000 and 3100 belong to other applications; this project uses 3200.

Demo accounts

All accounts below use **NurtureDemo!2026** in the generated local setup. If your environment was customized, use its configured `DEMO_PASSWORD`.

Email	What it lets you explore
worker@demo.nurturepath.test	Assigned Adenta records and care workflows
clinic@demo.nurturepath.test	Adenta clinic coordination, assignments and metrics
admin@demo.nurturepath.test	Facilities, access and programme aggregates
auditor@demo.nurturepath.test	Audit metadata and suppressed aggregates
worker2@demo.nurturepath.test	Assigned Tamale records; scope comparison
colleague@demo.nurturepath.test	Adenta worker with no initial assignments

These credentials are deliberately for the local fictional demo. Use unique private credentials if hosting a synthetic evaluation environment.

First setup on another computer

Install Node.js 24 and Docker Desktop with Linux containers. Copy the project source; keep keys and private environment files out of shared source archives.

```
npm ci
node scripts/setup-local.mjs
docker compose up -d db
npm run db:generate
npm run db:migrate
npm run db:seed
npm run dev
```

The setup script preserves an existing `.env`. The seed is idempotent and supplies two clinics, six demo users and twenty fictional mothers. Verify `http://localhost:3200/api/health` returns status `ok`.

02 / DAILY WORKER WORKFLOW

Take a record through the care loop

Sign in with the worker account. On a phone, use the navigation menu to reach all sections. This walkthrough uses invented adult details only.

1. Register a fictional adult

Use the enrolment action and complete the three-step registration form. Enter the fictional person's details, review the consent step, choose optional SMS/WhatsApp permissions, and submit. Confirm the record appears in **Mothers**. Essential enrolment consent and optional reminder consent are separate.

2. Schedule an appointment

Open the mother's record, enter the visit date and appointment details, then save. Scheduling creates a follow-up task automatically. Use **Appointments** to review planned visits.

3. Record the contact outcome

Open **Follow-ups** or the record's follow-up controls. Record reached, no answer, rescheduled or declined. For no answer and rescheduled, supply the next action date. Save and verify the updated task status.

4. Track a referral to closure

Create a referral from the mother's record. Use the permitted status actions to record acceptance and attendance, then close it with a coordination note. Each transition is validated and audited; closure should represent a confirmed coordination outcome in the demo.

5. Check the result

Review the record timeline and **Impact**. Sign out while connected, then sign in as the clinic coordinator. The clinic's closure metrics include the worker's completed referral. Small aggregate cells are suppressed to limit disclosure.

> **What “done” means in the demo**
> The record was saved, an appointment created its follow-up, the outcome was recorded, the referral completed its allowed transitions, and clinic metrics reflect closure.

Consent changes and archiving

Use the record's consent and archive actions when available to your role. Withdrawing reminder permission should prevent further simulation on that channel. Review confirmation text before archiving. Archiving is not an approved legal erasure process.

Workers see assigned records. A missing record may reflect assignment or clinic scope; ask the coordinator to check access rather than copying records into another account.

Explore the rest of the system

Role / section	Available use
Clinic coordinator	Coordinate care inside the clinic, review clinic impact and manage the clinic team/assignments through the available controls.
Programme admin	Manage facilities and access; view suppressed programme aggregates. Patient lists and care timelines are unavailable.
Auditor	Review audit metadata and suppressed aggregates. No contact details or patient record browsing.
Message library	Review approved source templates and deterministic drafts. A new draft requires review; it is not automatically approved or sent.
Audit trail	Inspect permitted action metadata. Logs avoid contact details, message bodies and clinical notes.

Simulate a reminder

1. Open an assigned record with the relevant reminder consent.
2. Choose the available neutral template/channel and preview the message.
3. Use the simulation action and inspect its recorded result.
4. Withdraw that channel's consent and verify a further simulation is rejected.

> **There is no live SMS or WhatsApp connection.** No reminder reaches a phone. The content assistant uses approved local sources and deterministic drafting; it does not call an external AI service or accept patient prompts.

Enable your two-factor sign-in

Use a dedicated test account so shared demo logins remain available. Open the account security controls, confirm your current password and start authenticator setup. Add the displayed secret to an authenticator app, enter its current code and complete activation. Subsequent sign-in needs password plus code. Codes cannot be reused after acceptance. Keep the authenticator available; self-service MFA recovery is not provided.

Enabling MFA revokes older sessions. Password resets and deactivation also invalidate stale sessions, including sign-ins racing with the change. Local recovery is a localhost development simulator; hosted verified recovery is still required.

Sign out safely

Reconnect before signing out. Successful sign-out revokes the server session and wipes the tab's queued offline changes, encryption key and in-memory records. If offline, the app asks you to reconnect so revocation can complete.

Capture offline, then synchronize

Offline support is designed around a prepared tab and an encrypted pending registration queue. It does not provide a complete offline database of all mothers.

1. **Prepare while connected.** Sign in and open `/offline` at the same localhost origin. Let the offline shell and static assets load.
2. **Keep the same tab.** Disable connectivity using browser developer tools to simulate offline mode. For this localhost demo, disabling Wi-Fi alone may leave the local server reachable.
3. **Test refresh.** Refresh offline after preparation. The public registration shell should load.
4. **Register a fictional adult.** Complete the form and submit. Verify it is queued locally; it is not yet a server record.
5. **Reconnect.** Use **Sync now** in the offline shell or the **Offline queue** section.
6. **Confirm the result.** Check that the queue clears and the new record appears in Mothers. Schedule its appointment after synchronization.

Limits you need to know

- Pending payloads are encrypted in IndexedDB. Their key stays in tab-session storage, and queued changes expire after 24 hours.
- Closing the tab, clearing browser storage or signing out can lose queued work or its key. Synchronize before doing those actions.
- Already-loaded records can remain in memory during a connection loss. Offline refresh supports registration; it does not restore previously loaded patient detail.
- Only public shell/static assets are cached. Personalized HTML and API responses are not stored in the service worker's cache.
- Synchronization rechecks authentication, consent, clinic/assignment scope and record version. Duplicate replay is handled idempotently.

If synchronization fails

Reconnect and confirm you are using the same account/tab. Read the error. If assignment, consent or version changed, refresh the authoritative record and reconcile with the coordinator. Discard obsolete queued content and re-enter the correct action where appropriate. Do not force an overwrite.

> **Before leaving a workstation**
> Connect → synchronize → verify records → sign out. A tab-held key is an additional control, not a substitute for device security or an approved lost-device process.

05 / DELIVERY CHECKPOINT

What has been completed

Area	Implemented
Product	Responsive landing/legal pages, original brand/PWA assets and worker, clinic, admin and auditor dashboards.
Care coordination	Consent-led adult enrolment, mother records, appointments, follow-up outcomes, referral transitions/closure, consent withdrawal and archiving.
Access and security	Tenant/clinic/assignment checks, role-specific data access, encrypted contacts/key versions, redacted audit, origin protection, rate limits, password hashing, opaque version-bound sessions and TOTP.
Offline and content	Encrypted actor-bound queue, expiry/idempotent replay, safe public shell, neutral reminder simulation and approved-source draft/review.
Delivery foundation	Database migration/seed, lockfile, CI configuration, standalone build, container configuration, local encrypted backup/restore tools, runbooks and screenshots.

What was actually verified

Check	Observed result
Unit/database tests	17 passed across four files
Desktop/mobile browser suite	12 passed in the final compiled-app run; 6.1 minutes
Lint / TypeScript / build	Passed; Next.js 16.4.0
Local encrypted restore	Eight table counts matched; 15.436 seconds; separate scratch database
Local synthetic load	100 reads, concurrency 5, zero failures; p95 1,036 ms exceeds the 800 ms target
Dependency review	Zero runtime advisories at review; five high development-only lint-chain findings remain tracked
Container build	Two attempts failed during downloads; ECONNRESET. No successful container smoke test
Extension hydration fix	Solvely attribute reproduced before hydration in Chrome; no hydration warnings. Lint/types passed.

The full suite preceded the final narrow layout fix; that fix received targeted Chrome verification plus lint/types. Evidence is in docs/VERIFICATION.md and the backup/load JSON files. These results do not prove production availability, clinical safety or independent security certification.

What remains, and how to finish it

1. Finish reproducible hosted-demo deployment

Owner: developer / deployment lead. Resolve Docker's registry connectivity or proxy issue and repeat `docker build -t nurturepath-demo:reviewed .`. Keep TLS verification enabled. Run the built image against a separate synthetic database, then verify health, sign-in and referral closure. Alternatively validate a managed Next.js deployment using docs/deployment.md.

Acceptance: immutable build, passing container/hosted smoke tests, TLS/exact origin, secure cookies, controlled migrations, unique secrets and tested rollback. No hosting account or public domain is currently provisioned.

2. Connect identity, keys and audit infrastructure

Owner: developer + security lead. Add verified recovery delivery and single-use recovery tests. Connect a managed key service to the versioned encryption keyring; exercise rotation and restored-backup decryption. Export redacted audit metadata to an access-controlled immutable store.

Acceptance: reviewed recovery abuse controls, access/key-rotation drills and tamper-evidence assessment. Do not replace the sole decryption key in place.

3. Build the live reminder pipeline

Owner: developer + partner/content owners. After approved provider arrangements, implement durable scheduled jobs, retry/backoff, deduplication, quiet hours, opt-out/STOP processing and authenticated delivery webhooks. Recheck consent immediately before every send; avoid clinical details in neutral copy.

Acceptance: provider-sandbox tests demonstrate consent withdrawal, retry safety, signed-webhook rejection, duplicate handling and opt-out. Keep the simulator active until release approval.

4. Close performance, accessibility and content gaps

Owner: developer + independent reviewers. Profile scoped reads and database plans, improve measured bottlenecks, and repeat a documented staging load test. Complete keyboard/screen-reader, mobile OS and device-storage assessment. Extend content governance with reviewer identity, source/version, approval history and qualified translation review.

Acceptance: p95 under 800 ms at an agreed load, resolved accessibility findings and versioned content approval. No current local-language publication or validated clinical translation is claimed.

> **Recommended order**
> Working hosted synthetic demo → identity/key/audit infrastructure → provider sandbox and content governance → independent tests and exercises → written gates → monitored pilot.

Obtain evidence before real onboarding

Every gate below is pending. The final production PDF requires external decisions and operational evidence that software alone cannot supply.

Gate / owner	How to complete it	Evidence to retain
A — Partner Partner/legal lead	Confirm company rights, clinic authorization and controller/processor responsibilities with the participating organisations.	Signed authorizations and processing agreements
B — Legal Qualified counsel / controller	Review the data inventory, applicable law, residency/transfers, consent, retention and rights process; complete the required privacy assessment.	Written legal review, approved policies and DPIA/consent decisions
C — Clinical Qualified clinician	Review workflows, referral rules, emergency guidance, neutral copy and any translations. Record approvals against specific versions.	Clinical/content sign-off and controlled change process
D — Security Security owner	Commission an independent access/threat assessment. Fix findings; exercise encrypted restore on approved staging and review key/device controls.	Assessment, remediation/retest and recovery evidence
E — Operations Operational owner	Name on-call/support owners; establish monitoring, escalation and service targets; rehearse outage, incident and lost-device processes.	Runbooks, measured SLOs, drill results and contacts
F — Pilot Partner + clinical + legal	Agree a limited cohort, consent process, monitoring, stop criteria and rollback, then approve the reviewed release.	Signed pilot protocol and release authorization

Then release a limited pilot

1. Review all six approvals and remaining findings together.
2. Approve hosting/database/provider contracts and separate environments.
3. Test backup, migration, rollback, monitoring and the same immutable release artifact.
4. Implement a reviewed onboarding-enabled release after evidence is verified.
5. Launch the agreed limited pilot, monitor results and stop if its criteria are triggered.

> **Changing DEMO_MODE is not approval.** In this release, `DEMO_MODE=false` blocks care writes. Do not bypass that control to enter real records. Real-data launch requires a subsequent reviewed software release.

This guide describes project completion steps; it does not provide medical advice, legal conclusions, certification or a claim of UNICEF endorsement.

Keep the demo working

Routine checks

```
npm run lint
npm run typecheck
npm test
npm audit --omit=dev
```

Database tests need the migrated/seeded local database. For a release check, stop the server first on Windows, then build and start:

```
npm run build
npm run start
# In a second PowerShell terminal:
$env:PLAYWRIGHT_CHANNEL='chrome'
$env:PLAYWRIGHT_PRODUCTION='true'
npm run test:e2e
```

Browser tests leave clearly labelled fictional E2E records. The guarded reset command is documented in README.md; it clears local demonstration care records, so review its confirmation requirements first.

Common problems

Symptom	Action
Site unavailable	Start the app; read the terminal output; use port 3200. Check Docker Desktop and `docker compose up -d db`.
Sign-in rejected	Check account/password, exact APP_URL origin and any enabled authenticator code. Use the configured demo password if changed.
Record missing / forbidden	Confirm worker assignment, clinic, role, consent and account status with the coordinator.
Offline queue will not sync	Reconnect in the same tab/account; inspect conflicts; synchronize before logout or closing the tab.
Build reports Prisma DLL locked	Stop the project's running Node server with Ctrl+C, then rebuild. Do not stop unrelated apps.
Extension hydration warning	Refresh after the applied root-layout fix. The reported Solvely attribute was verified. A different warning needs its own diagnosis.

Files to hand to the next developer

README.md: setup/accounts. **docs/CURRENT-STATE.md:** continuation checkpoint. **docs/VERIFICATION.md:** measured evidence. **docs/PRODUCTION-READINESS.md:** remaining gaps. **docs/deployment.md** and **docs/operations.md:** release/recovery guidance. **docs/architecture.md** and **docs/api.md:** technical map. Keep environment keys and encrypted backups separate from source control.

Editable guide: docs/USER-GUIDE.html. Recreate the PDF with `node scripts/create-user-guide.mjs` using installed Chrome. Source PDFs remain in the project root.